



Population



68,250

Growth Rate



3.00%



Outpatient visits



1.78

Number of outpatient visits per person per year - a key metric in service delivery. The best districts provided 2+ visits per person per year

Facility Births



60.6%

Percentage of women who give birth in a health facility - one of the most important interventions to prevent maternal and newborn deaths

Antenatal Care



59%

Percentage of pregnant women attending a first antenatal care checkup - these visits help to identify and treat any issues during pregnancy and prepare women and their families for a safe child birth

Family Planning



162

Couple years protection provided per 1000 women aged 15-44 years. Districts should be aiming for 300 or more. This allows individuals and couples to anticipate and attain their desired number of children and the spacing and timing of their births

Low Birth Weight



5.4%

Percentage of babies born in health facilities weighing less than 2500g - Low birth weight contributes to 60-80% of neonatal deaths, and increases risk of infections and developmental disorders

Malnutrition



9.5%

Percentage of children under 5 years who are underweight for their age - malnutrition is estimated to contribute to more than a third of childhood deaths

Measles Vaccination



55.7%

Percentage of children under 1 year vaccinated against measles - Countries aiming at measles elimination should achieve ≥95% coverage with both doses in every district

Outreach Clinics



38

Number of outreach clinics per 1000 children - rural outreach provides a key platform for preventive child health programs and districts should be achieving over 50

Pneumonia Deaths



Percentage of children under 5 years admitted to health facilities with pneumonia who die - good quality care (oxygen, early and effective use of antibiotics) would reduce these deaths. Pneumonia can be prevented by immunization, adequate nutrition (including exclusive breastfeeding), and by addressing environmental factors. Pneumonia should be treated with antibiotics

DISTRICT PRIORITIES



Improve hospital and rural health services



Improve partnerships with churches and private sector



Get more health workers to meet population needs



Make sure women have access to and give birth in a health facility



Health worker education, and increasing the number of funded positions for health care workers. Investing in the primary health care workforce is the most cost-effective way to improve access to essential health care



Increase the number of children immunised against diseases



Refurbish and upgrade poor and unsafe infrastructure



Expand services to address high levels of family violence



Increase family planning options: children by choice, not chance



Improve monitoring of supply of medicines and health technologies to facilities



Full adoption of the eNHIS health information system to ensure up to date and accurate information

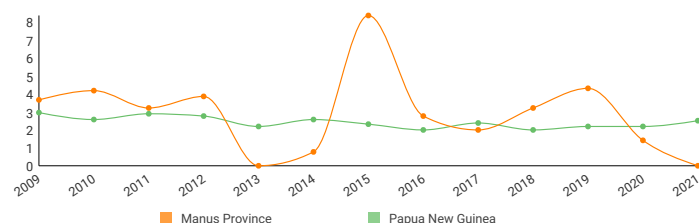
Why invest DSIP funds in health?

The district is the frontline in service delivery. The money you invest in health can and does make a real difference to health outcomes. The money can be directed to where it is needed most - whether that is fuel for outreach visits or rebuilding aid posts. These investments save lives and mean all families can access quality services to live happy and healthy lives.

For further information, contact your provincial health officer.

INDICATOR 1

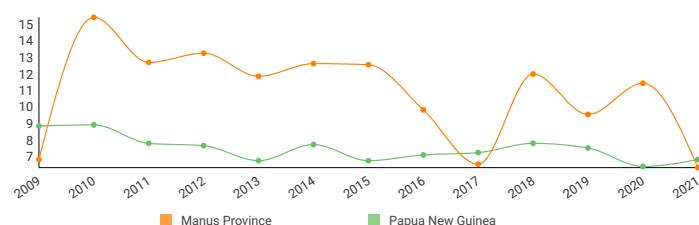
Percentage of pneumonia admissions in children aged less than 5 years that die in facilities



The percentage of children under five years of age that are admitted to a health centre with pneumonia and die during that admission. Good quality care (oxygen, early and effective use of antibiotics) would minimise these deaths.

INDICATOR 3

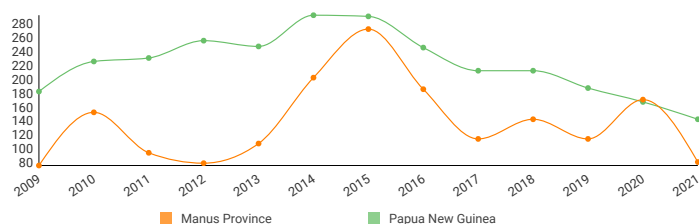
Percentage of Low Birth Weight Babies



The percentage of live births in health facilities that weigh less than 2500 grams (low birth weight).

INDICATOR 6

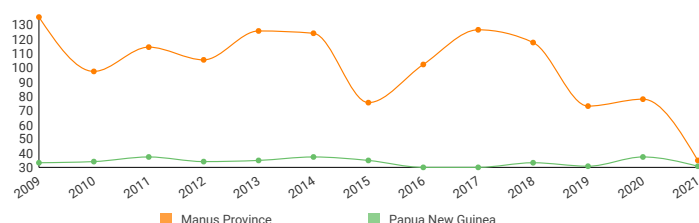
Incidence of Diarrhoeal Diseases in Children under 5 years



This indicator measures the number of children under 5 years treated as outpatients at a health facility for diarrhoeal illness per 1000 children under five years in the population. Diarrhoeal illness serves as an indicator of water quality, food hygiene and personal hygiene.

INDICATOR 8

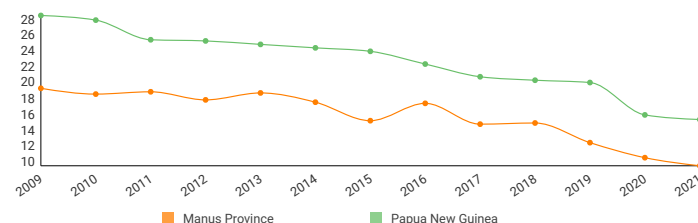
Outreach Clinics per 1000 children under 5 years



This indicator measures the number of outreach clinics (including static clinics, day mobile clinics, overnight/patrol clinics) held per 1000 children under 5 years old in the population. Rural outreach provides the key platform for preventive child health programs, and an opportunity for individual community health education.

INDICATOR 2

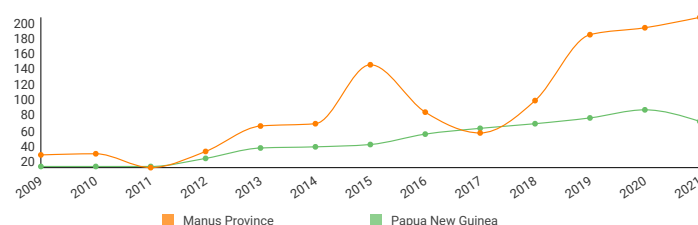
Percentage of Children with Moderate and Severe Malnutrition



Percentage of children under 5 years who attend Maternal and Child Health clinics that are moderately or severely malnourished. Before September 2019, moderately malnourished was measured as 60 – 80% weight for age, and severely malnourished was measured as <60% weight for age. Since September 2019, moderately malnourished is measured by z scores between -3 and -2, and severely malnourished is measured by z scores less than -3.

INDICATOR 4

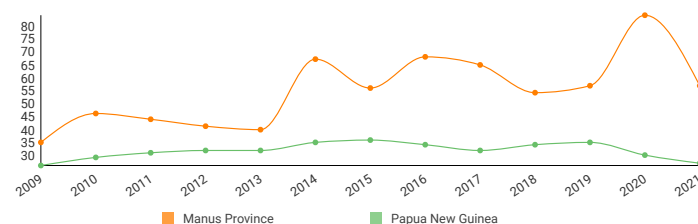
Malaria Incidence per 1000 people



The rate of confirmed cases of malaria (confirmed using Rapid Diagnostic Tests (RDT) or by blood samples tested on microscope slides) per 1000 people in the population. Unconfirmed cases are not included. In 2013, the RDT to confirm a malaria diagnosis was adopted for widespread use at health facilities in PNG. Prior to that, confirmed cases of malaria could only be detected using microscope slides which were often unavailable or otherwise unused.

INDICATOR 7

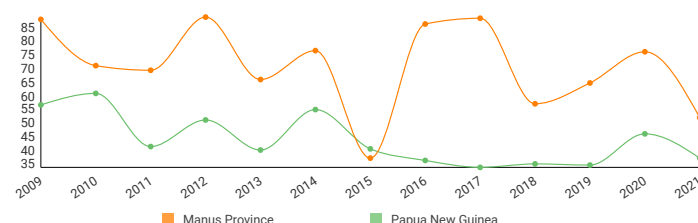
Injury Presentations for every 1000 people



This indicator measures the number of people treated as outpatients at a health facility for injuries per 1000 people in the population.

INDICATOR 9A

Percentage of children under 1 year that are vaccinated for measles

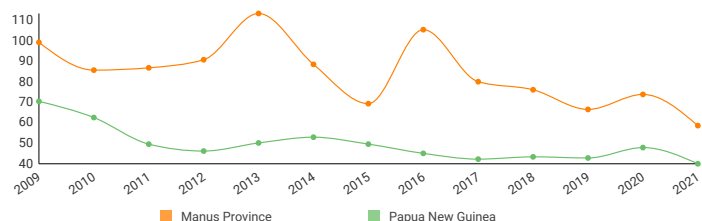


Immunisation is an essential component for reducing child mortality. Immunisation coverage estimates are used to monitor coverage and quality of child care services throughout the country. Measles is the leading cause of childhood mortality from vaccine-preventable diseases. The indicator provides a good measure of health system performance.



INDICATOR 9B

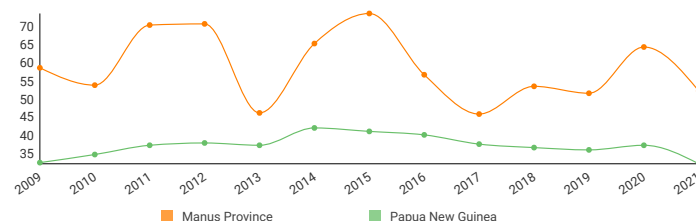
Percentage of children under 1 year receiving three doses of Pentavalent Vaccine



This indicator measures the percentage of children under 1 year who have received three doses of DTP-Hib - HepB (Pentavalent) vaccine. Prior to 2009, TA (DTP) was provided rather than the pentavalent vaccine.

INDICATOR 10A

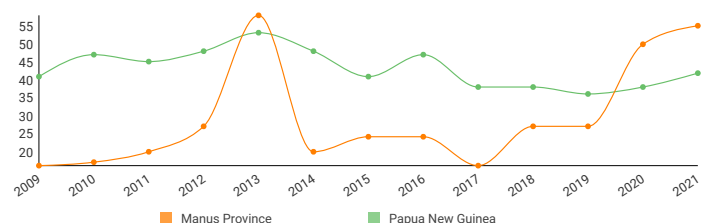
Percentage of Supervised Births at Health Facilities



This indicator looks at the percentage of births at health facility or village attended by skilled health personnel. Measuring maternal mortality is unusually difficult, and the current method (sisterhood method) cannot be used to provide short term trends. The supervision of delivery is hence used as a proxy for this purpose.

INDICATOR 10B

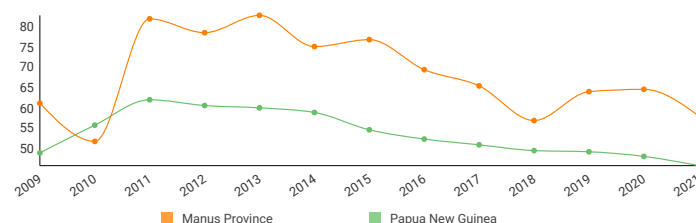
Referred rate per 1000 births



This indicator lists the number of women transferred to hospital due to pregnancy or delivery complications for every 1000 births in a health facility.

INDICATOR 11

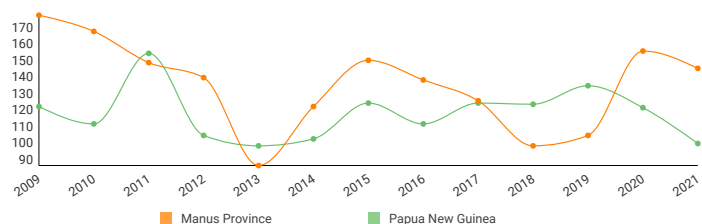
Antenatal Coverage



Antenatal care is an indicator of access to and use of health care during pregnancy. The indicator shows the percentage of pregnant women that attended a first antenatal visit at hospital, health centre or outreach clinic.

INDICATOR 12

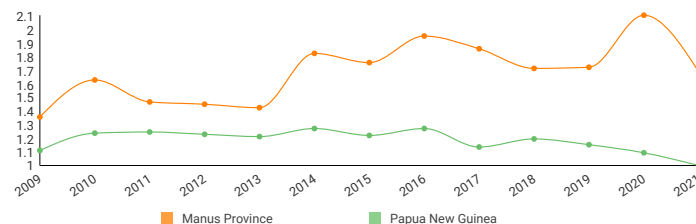
Family Planning Use



The indicator shows the family planning measure "Couple Years Protection" for modern contraceptive methods per 1000 women of reproductive age. Couple Years Protection (modern) is a measure of how much protection is provided by modern family planning methods in the population. It includes oral contraceptive pills, IUDs, implants, condoms, vasectomies, tubal ligations, and injectable contraceptives.

INDICATOR 21

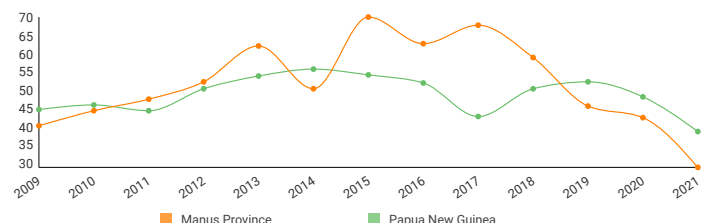
Outpatient visits per person per year



This indicator measures the number of hospital and health centre outpatient visits per year. It is assumed that the more accessible the health facility in terms of location, staffing etc, the more likely people will seek to use it.

INDICATOR 27

Availability of Medical Supplies



This indicator monitors the percentage of months in a year that have no shortages of 8 essential medical supplies. A value of 100% means that there was no shortage of essential supplies during the year, a value of 0% means that there was a shortage of essential medical supplies during every month in the year.